

Transition is the end of active labor as your cervix becomes fully dilated and you ready yourself to begin pushing. This is one of the shortest stages of labor, lasting from 15 minutes to two hours, although on average it's about 30 minutes. This can be one of the most challenging parts of labor because your contractions intensify and can begin to feel continuous since they now occur every 30–90 seconds. If you haven't had an epidural, transition can be especially difficult since you may feel a lot of pressure on your lower back and rectum and have an overwhelming desire to push, but will be unable to push until your cervix is fully dilated. Even if you've had an epidural, you may notice increasing pelvic pressure. If you do push before your cervix is ready, you may tear your cervix or cause your cervix to swell and thicken, which will prolong the process of labor.

It's not uncommon to vomit now, a side effect of the stretching of your cervix and the pelvic pressure. You may also tremble or shake and have hot flashes.

How you can cope You may feel very uncomfortable during this period as your contractions become stronger and you try to hold back from pushing. Significant pelvic pressure during the transition phase can make it difficult to relax between contractions, and you will therefore need plenty of support from your birth partner and midwife. You may be feeling exhausted, out of control, possibly frightened, and may even think that you can't continue.

Work with your nurse to find the best position for you. This is the one stage of labor where it can be helpful not to adopt an upright position, since you can take some of the pressure off the pelvis. Sitting or being on all fours with your bottom raised may help. Keep breathing during your contractions; your nurse may show you how to pant and breathe shallowly to help resist the urge to push. If possible, moving around during contractions can sometimes help since you will focus on doing something else until you can actively push. You could try rocking on a birthing ball or in a rocking chair. If there is time between your contractions, ask your partner to massage your lower back if this helps relieve pressure.

It can be easy to lose sight of the purpose of labor at this point, so try to focus on the fact that your baby will soon be born.

Pain relief Your doctor may not give you opioid pain relief now, since it can cause your baby to be too sleepy if it is given close to the birth. You may still be able to have an epidural.

SUPPORT IN THE FIRST STAGE

During the first stage, your birth partner has a varied and important role. In addition to helping you feel comfortable and assisting you with positions, they can also support you mentally. This is especially important at the end of the first stage, when you reach [transition](#), a point when women often feel panicky and out of control. Your birth partner can offer reassurance that you're doing well and that the delivery of your baby is not far off. They can also improve your comfort, for example by applying a damp, cool washcloth to your face and neck, and can help you focus on your breathing, reminding you to pant or blow to help you resist the urge to push before the cervix is fully dilated.